

1. Administrative & Study Identification

Full Study Title: A Study of Nivolumab in Combination With Ipilimumab in Chinese Participants With Previously Treated Advanced or Recurrent Solid Tumors **Short Title/Acronym:** Phase 1/2 Nivolumab + Ipilimumab in Chinese Solid Tumors **Protocol Number:** CTR20170731 **NCT Number:** NCT03195478 **Posting ID:** 63672354818440070 **Sponsor/Company Name:** Bristol Myers Squibb **Investigational Product:** Nivolumab (BMS-936558) and Ipilimumab (BMS-734016) **Phase of Development:** PHASE1, PHASE2 **Medical Monitor:** Clinical Operations Lead, Bristol Myers Squibb

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the safety and effectiveness of Nivolumab in combination with Ipilimumab in Chinese participants with previously treated late-stage cancer. **Secondary Objectives:** To characterize the pharmacokinetic (PK) profile (including Cmax, Tmax, and AUC(0-T)) and evaluate the preliminary anti-tumor efficacy of the combination therapy in the target population.

2.2 Background & Rationale To address the gap in the pharmacokinetic (PK) and pharmacodynamic (PD) profile data of immune checkpoint inhibitors specifically within the Chinese population. While immunotherapy is a standard of care globally, understanding the safety, tolerability, and efficacy of the dual blockade of PD-1 and CTLA-4 (nivolumab plus ipilimumab) ensures optimal dosing regimens for Chinese participants with advanced or recurrent solid tumors who have failed prior therapies.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Single-arm (Dose-escalation and Dose-expansion) **Blinding:** Open-label **Randomization:** N/A **Trial Status:** COMPLETED

3.2 Planned Enrollment & Duration Targeted Enrollment: 37 **Number of Sites:** Multiple centers in China **Estimated Study Start:** August 2017 **Primary Completion:** Study has been completed.

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Mainland Chinese participants with advanced or recurrent solid tumors. 2. Eastern Cooperative

Oncology Group (ECOG) performance status of 0 or 1. 3. One prior anti-cancer therapy that did not work or documented refusal to receive chemotherapy or biological therapy.

4.2 Exclusion Criteria 1. Cancer that has spread to the brain or central nervous system unless it has been adequately treated. In addition, either no longer receiving corticosteroids, or on a stable or decreasing dose of no more than 10 mg daily prednisone (or equivalent). 2. Active, known or suspected autoimmune disease or infection. 3. Positive blood screen for chronic infection of hepatitis B or hepatitis C (HCV antibody positive unless HCV RNA is negative). 4. Prior immuno-oncology therapy. (*Other protocol-defined inclusion/exclusion criteria apply*)

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: * **Cohort A:** Nivolumab (BMS-936558) 3 mg/kg once every 2 weeks + Ipilimumab (BMS-734016 / Trade Name: Yervoy, ATC: L01FX04, EMA URL: https://www.ema.europa.eu/en/documents/product-information/yervoy-epar-product-information_en.pdf) 1 mg/kg once every 6 weeks. * **Cohort B:** Nivolumab 3 mg/kg once every 3 weeks + Ipilimumab 1 mg/kg once every 3 weeks.

Route of Administration: Intravenous (IV) infusion **Duration of Treatment:** Treatment continues until disease progression, unacceptable toxicity, or withdrawal of consent.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care medications (e.g., anti-emetics, pain management). **Prohibited:** Concurrent systemic immunosuppressive therapy (e.g., corticosteroids ≥ 10 mg/day prednisone equivalent unless specified for treated brain metastases), other investigational anti-cancer agents, or live-attenuated vaccines.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Baseline Tumor Imaging, Viral Screening Labs
Baseline	Day 0	Enrollment, First Dose of Nivolumab + Ipilimumab, Baseline PK
Interim	Every 2-3 Weeks	Safety Labs, PK Blood Draws, Efficacy Assessment (Tumor Imaging every 6-8 weeks)
End of Study	Up to 100 days post-last dose	Final Vital Signs, Safety Follow-up, Survival Status, IP Accountability

7. Outcome Measures (Endpoints)

7.1 Primary Outcomes 1. Number of Participants With Adverse Events (AEs) in Part 1. 2. Number of Participants With Serious Adverse Events (SAEs) in Part 1. 3. Number of Participants With Adverse Events Leading to Discontinuation in Part 1.

7.2 Secondary Outcomes 1. **C_{max}**: Maximum Observed Serum Concentration in Part 1. 2. **T_{max}**: Time of Maximum Observed Serum Concentration in Part 1. 3. **AUC(0-T)**: Area Under the Plasma Concentration-time Curve in Part 1.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring via clinical and laboratory evaluations. Special focus is placed on Immune-Mediated Adverse Events (imAEs) such as pneumonitis, colitis, hepatitis, and endocrinopathies. **Serious Adverse Events (SAEs):** To be reported to the sponsor and regulatory authorities within 24 hours of investigator awareness. **Data Monitoring Committee (DMC):** An internal Safety Review Committee (SRC) evaluated DLTs and made dose-escalation decisions.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: * **Safety Set:** All patients who receive at least one dose of the study drug combination. * **DLT-evaluable Population:** Used specifically for dose-escalation tier decisions. **Sample Size Justification:** A standard dose-escalation cohort design utilizing small sample sizes scaled dynamically according to the toxicities observed, leading to a targeted enrollment of 37 participants. **Handling of Missing Data:** Safety data will be summarized with descriptive statistics without imputation. Efficacy parameters will be evaluated based on observed data.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study was conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Regular onsite and remote monitoring to verify source data, ensure protocol adherence, and track safety events. **Audit Trail:** Electronic Case Report Forms (eCRFs) featuring a complete audit trail; data clarification processes implemented prior to database lock.

11. Study Identifiers & Governance

Primary ID: CTR20170731 **Registry Number:** NCT03195478 **Posting ID:** 63672354818440070 **Sponsor/Lead Organization:** Bristol Myers Squibb
Study Title: A Study of Nivolumab in Combination With Ipilimumab in Chinese Participants With Previously Treated Advanced or Recurrent Solid Tumors **Official Regulatory Title:** A Phase 1/2 Study of Nivolumab (BMS-936558) in Combination With Ipilimumab (BMS-734016) in Chinese Participants With Previously Treated Metastatic or Recurrent Solid Tumors **Trial Status:** COMPLETED

12. Clinical Framework (Solid Tumor Specific)

Primary Condition: Solid Tumor (Advanced or Recurrent Solid Tumors) **Diagnosis Threshold:** Histologically or cytologically confirmed metastatic or recurrent solid tumor that has progressed after standard systemic therapies. **Medical Methodology:** Immune Checkpoint Blockade therapy (anti-PD-1 combined with anti-CTLA-4). **Optimization Target:** Identification of the Maximum Tolerated Dose (MTD) / Recommended Phase 2 Dose for Chinese patients and tracking of specific Adverse Events.

13. Study Procedures & Methodology

Management Pathway: Standard oncology clinical pathway incorporating combination immunotherapy. **Education Protocol (HCP):** Site initiation visits covering study drug preparation/administration, immune-related AE (imAE) management algorithms, and rigorous PK sampling protocols. **Education Protocol (Patient):** Informed consent processes detailing immune-related side effects and instructions on emergency symptom reporting. **Quality Audit Mechanism:** Regular central safety review and continuous data monitoring to oversee dose-escalation thresholds.

14. Observation & Follow-Up Schedule

Total Duration: Throughout the treatment duration until disease progression or unacceptable toxicity, followed by survival tracking. **Onsite Visit Cadence:** Every 2 to 3 weeks, mapped sequentially to the selected IV infusion schedule (Cohort A or B). **Remote Monitoring Frequency:** Interim safety and AE reporting check-ins as required. **Checklist Review Interval:** Tumor imaging and RECIST v1.1 efficacy assessments conducted every 6 to 8 weeks.

15. Sign-off & Approval

	Role		Name		Signature		Date			:---		:---		:---		:---	
	Principal Investigator		[Insert Name]		_____		[DD-MMM-YYYY]										
	Clinical Operations Lead		[Insert Name]		_____		[DD-MMM-YYYY]										
		Lead Statistician		[Insert Name]		_____		[DD-MMM-YYYY]									